

Behavioral Emergency / Intoxication

History • Ingestion of substances / alcohol? ◦ What, when, how much? • Was there any trauma? • Does the patient have history of psychiatric disorder or take psychiatric medications? • Patient admitted to psychiatric facility in the past? • Is patient suicidal, homicidal or hallucinating? • Was there domestic violence involved? • Patient complaining of chest pain or headache? • What are the patient's other medical problems?	Physical • General appearance – Calm? Agitated? Comatose? Violent? • Vital Signs – include temperature and SpO2 • Mental status – Awake and alert? Confused? Lethargic? • Respiratory effort – Increased? Decreased? Wheezing? • EKG – Cardiac dysrhythmias? STEMI? • Trauma – Signs of injury? Abuse? Suicide attempt? Illness / Injuries that can present as behavioral emergencies • Life-threatening intoxication • Infection, sepsis, meningitis, encephalitis • Intracranial hemorrhage • Traumatic injury • Liver failure • Kidney failure • Diabetic Ketoacidosis (DKA) • Hypoglycemia • Severe electrolyte disturbances • Excited Delirium
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EMT/AEMT

- Follow General Medical Care Guideline

Extreme Agitation / Danger to Self or Others

- Never allow a restrained patient to be kept prone or face down at any time
- If restraints are needed, soft restraints to limbs only with neurovascular checks every 5 minutes
- Backboard use for immobilization is not recommended
- Secondary assessment
- Blood sugar check, Vital Signs

Paramedic

- Follow General Medical Care Guideline
 - Place SpO2 and ETCO2 (as available) on all patients even if not sedated

Anxiety

- **Hydroxyzine** 25-50mg PO (if available)
- May repeat in 6 hours as needed
- Sedating - do not drive, operate heavy machinery, or engage in firefighting operations

Psychosis without immediate danger

- **Droperidol** 5mg IM
- May repeat once in 20 min if desired effect not achieved
- Sedating - do not drive, operate heavy machinery, or engage in firefighting operations

Extreme Agitation / Danger to Self or Others

- **Droperidol** 10mg IM
- Call medical direction for further orders

Dystonic Reaction after Droperidol

- **Diphenhydramine (Benadryl)** 25-50 mg IV / IM / PO slowly

****Ventilation and cardiac monitoring are required for all sedated patients****
 Do not give hydroxyzine, benadryl, and droperidol together - **choose one**